

9109433



CYCLOGYL™

1% Sterile Ophthalmic Solution

1 NAME OF THE MEDICINAL PRODUCT
CYCLOGYL™ 1% Sterile Ophthalmic Solution

2 QUALITATIVE AND QUANTITATIVE COMPOSITION
1 mL solution contains 10 mg cyclopentolate hydrochloride
For excipients, see section 6.1.

3 PHARMACEUTICAL FORM
Eye drops, clear sterile colorless solution

4 CLINICAL PARTICULARS
4.1 Therapeutic Indications
CYCLOGYL™ (Cyclopentolate Hydrochloride) is used chiefly for mydriasis and cycloplegia in diagnostic procedures.

4.2 Posology and Method of Administration
Adults and elderly:
Instill 1 or 2 drops of 1% solution in the eye which may be repeated in 5 to 10 minutes. Complete recovery usually occurs in 24 hours. Complete recovery from mydriasis in some individuals may require several days.

Pediatrics:
Children:
Instill 1 or 2 drops of 1% solution in the eye which may be repeated in 5 to 10 minutes later by a second application of 1% solution if necessary.

CYCLOGYL™ should not be used in small infants as concentrations greater than 0.5% are not recommended due to the risk of serious systemic side effects.

The lowest concentration and number of drops necessary to produce the desired effect should always be used. If any medicine misses the eye, wash away immediately.

Elderly population
Elderly patients may be a higher risk for undiagnosed glaucoma as well as cyclopentolate-induced psychotic reactions and behavioral disturbances (see Section 4.4).

Hepatic and renal impairment
No formal studies have been conducted in patients with renal or hepatic impairment.

Method of Administration
For ocular use only.
After cap is removed, if tamper evident snap collar is loose, remove before using product.
Nasolacrimal occlusion or gently closing the eyelid after administration is recommended. This may reduce the systemic absorption of medicinal products administered via the ocular route and result in a decrease in systemic adverse reactions.

4.3 Contraindications

- Hypersensitivity to the active substance or to any of the excipients.
- Patients with known or suspected angle-closure glaucoma.

4.4 Special Warnings and Precautions for Use

- Use with caution in patients, especially children, who have previously had a severe systemic reaction to atropine. CYCLOGYL™ may cause increased intraocular pressure. The possibility of undiagnosed glaucoma should be considered in some patients, such as elderly patients. Determine the intraocular pressure and an estimation of the depth of the angle of the anterior chamber prior to initiation of therapy to avoid glaucoma attacks (see Section 4.8).
- Cyclopentolate-induced psychotic reactions and behavioral disturbances and other CNS disturbances may occur in patients with increased susceptibility to anticholinergic drugs (see Section 4.8). Use with caution in children and elderly patients, but reactions may occur at any age.
- Because of the risk of provoking hyperthermia, use with caution in patients, especially children, who may be exposed to elevated environmental temperatures or who are febrile.
- Patients may experience sensitivity to light and should protect eyes in bright illumination (see Section 4.8).
- CYCLOGYL™ contains benzalkonium chloride which may cause eye irritation and is known to discolor soft contact lenses. Avoid contact with soft contact lenses. Patients must be instructed to remove contact lenses prior to application of CYCLOGYL™ and wait 15 minutes before reinsertion.

Pediatric population

- Use with extreme caution, if at all, in infants, small or premature children, or children with Down syndrome, spastic paralysis or brain damage (see section 4.2).
- Premature and small infants, young children, or children with Down syndrome, spastic paralysis or brain damage are particularly susceptible to central nervous system disturbances, cardiopulmonary and gastrointestinal toxicity from systemic absorption of cyclopentolate (see Section 4.8).
- Seizures and acute psychosis induced by cyclopentolate are especially prominent in children (see Section 4.8). Cyclopentolate should be used with caution in children with known epilepsy.
- Fair-skinned children with blue eyes may exhibit an increased response and/or increased susceptibility to adverse reactions.
- Observe infants closely for at least 30 minutes following instillation.
- Feeding intolerance (see Section 4.8) and necrotizing enterocolitis (NEC) in preterm infants may follow ophthalmic use of this product in infants. Cases of NEC have been reported in preterm infants following administration; however, causality has not been established. It is recommended that feeding be withheld for 4 hours after examination.
- Parents should be warned not to get this preparation in their child's mouth or on their child's cheeks and to wash their hands and the child's hands or cheeks following administration.

4.5 Interaction with Other Medicinal Products and Other Forms of Interaction
The effects of CYCLOGYL™ may be enhanced by concomitant use of other drugs having antimuscarinic properties, such as amantadine, some antihistamines, phenothiazine antipsychotics, and tricyclic antidepressants.

4.6 Fertility, Pregnancy and Lactation

Fertility
Studies have not been performed to evaluate the effects of topical ocular administration of cyclopentolate hydrochloride (CYCLOGYL™) on fertility (See Section 5.3).

Pregnancy
There are no or limited amount of data from the use of CYCLOGYL™ in pregnant women.

Breast-feeding
It is not known whether cyclopentolate/metabolites are excreted into human milk. A risk to the suckling child cannot be excluded.

4.7 Effects on Ability to Drive and Use Machines
Cyclopentolate may cause drowsiness, blurred vision and sensitivity to light. Patients receiving CYCLOGYL™ should be advised not to drive or engage in other hazardous activities unless vision is clear.

4.8 Undesirable Effects
Tabulated list of adverse reactions
The following adverse reactions have been identified from post-marketing surveillance following administration of CYCLOGYL™. Frequency cannot be estimated from the available data. Within each SOC, undesirable effects are presented in decreasing order of seriousness.

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By Aranda, Francisco

1:10 pm, Mar 31, 2025

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PACKAGING

FILE NAME: 9109433-0325 I CYCLOGYL 1% 15ML MAL

ARTIST: Aranda, Francisco

ART REV: A

DATE: 31.03.2025

SPECIAL INSTRUCTIONS: SPECIAL INSTRUCTIONS GO HERE

COLORS: Process Black

VENDORS PLEASE READ BELOW

COATING = 38 – 60 gloss units as measured by BYK-Gardner Micro-Gloss 60° gloss meter or similar device.
HIGH GLOSS COATING = No less than 75 gloss units as measured by BYK-Gardner Micro-Gloss 60° gloss meter or similar device.
MATTE COATING = No greater than 25 gloss units as measured by BYK-Gardner Micro-Gloss 60° gloss meter or similar device.

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System Organ Class	MedDRA Preferred Term (v 25.1)
Immune system disorders	hypersensitivity
Psychiatric disorders	hallucination, confusional state, disorientation, agitation, restlessness
Nervous system disorders	incoherent, retrograde amnesia, dizziness, headache, somnolence
Eye disorders	photophobia, eye pain, mydriasis (prolonged drug effect), eye irritation, vision blurred, ocular hyperaemia
Gastrointestinal disorders	vomiting, nausea, dry mouth
Skin and subcutaneous tissue disorders	erythema
General disorders and administration site conditions	gait disturbance, pyrexia, fatigue

Description of selected adverse reactions

This drug produces reactions similar to those of other anticholinergic drugs. The central nervous system manifestations such as ataxia, incoherent speech, restlessness, hallucinations, hyperactivity, seizures, disorientation as to time and place, and failure to recognize people are possible. Other toxic manifestations of anticholinergic drugs are skin rash, abdominal distention in infants, unusual drowsiness, tachycardia, hyperpyrexia, vasodilation, urinary retention, diminished gastrointestinal motility, and decreased secretion in salivary and sweat glands, pharynx, bronchi and nasal passages. Severe reactions are manifested by hypotension with rapid progressive respiratory depression.

CYCLOGYL™ may increase intraocular pressure and provoke glaucoma attacks in patients predisposed to acute angle closure in particular geriatric patients (see Section 4.4).

The onset of cyclopentolate toxicity occurs within 20 to 30 minutes of drug instillation, and although usually transient (subsiding in 4 to 6 hours), the symptoms can last 12 to 24 hours.

Pediatric population

Increased risk for systemic toxicity has been observed in premature and small infants, young children, or children with Down syndrome, spastic paralysis or brain damage with this class of drug (see Section 4.4).

Use of CYCLOGYL™ has been associated with psychotic reactions and behavior changes in pediatric patients. Central nervous system reactions manifest similar to those listed above. Seizures and acute psychosis induced by cyclopentolate are especially prominent in children.

Feeding intolerance may follow ophthalmic use of the product in infants (see Section 4.4).

A local or generalized allergic-type response to cyclopentolate consisting of an urticarial rash has been described in children.

4.9 Overdose

Systemic toxicity may occur following topical use, particularly in children. It is manifested by flushing and dryness of the skin (a rash may be present in children), blurred vision, a rapid and irregular pulse, fever, abdominal distension in infants, convulsions and hallucinations and the loss of neuromuscular coordination. Severe intoxication is characterized by central nervous system depression, coma, circulatory and respiratory failure, and death. Treatment is symptomatic and supportive. In infants and small children the body surface must be kept moist.

5 PHARMACOLOGICAL PROPERTIES

5.1 Pharmacodynamic Properties

Pharmacotherapeutic group: Ophthalmologicals, mydriatics and cycloplegics, anticholinergic agents ATC code: S01FA04

This anticholinergic preparation blocks the responses of the sphincter muscle of the iris and the accommodative muscle of the ciliary body to cholinergic stimulation, producing pupillary dilation (mydriasis) and paralysis of accommodation (cycloplegia). It acts rapidly, but has a shorter duration than atropine. Maximal cycloplegia occurs within 25 to 75 minutes after instillation. Complete recovery of accommodation usually takes 6 to 24 hours. Complete recovery from mydriasis in some individuals may require several days. Heavily pigmented irides may require more doses than lightly pigmented irides.

5.2 Pharmacokinetic Properties

Absorption

Following topical ocular administration, cyclopentolate is absorbed into the eye as well as the systemic circulation. Following topical administration of 2 drops of 1% cyclopentolate to the eye of patients undergoing cataract surgery, aqueous humor drug concentrations during surgery (55 to 125 minutes postdose) ranged from 1410 to 25,361 nM. The corresponding plasma drug concentrations over this same interval ranged from 1.03 to 7.55 nM. In healthy female volunteers administered 1 drop of 1% cyclopentolate to each eye, a mean maximum plasma drug concentration of 2.06 ± 0.86 nM was achieved within 1 hour. In another study, plasma cyclopentolate concentrations were determined following two 30-microliter unilateral doses of 1% cyclopentolate administered 5 minutes apart. Peak plasma drug concentrations ranged from 3.3 to 15.5 ng/mL (mean: 8.3 ± 4.1 ng/mL) and were achieved within 5 to 15 minutes following the second dose.

Distribution

Aside from the aqueous humor data reported above for cataract patients, the ocular and systemic distribution of cyclopentolate has not been reported.

Biotransformation

The metabolic pathways of cyclopentolate have not been reported in the literature.

Elimination

The elimination mechanisms of cyclopentolate have not been reported in the literature.

Linearity/non-linearity

The dose proportionality of cyclopentolate has not been reported in the literature.

Pharmacokinetic/pharmacodynamic relationship(s)

The maximum pupillary dilation in female volunteers administered 1% cyclopentolate bilaterally occurred at 30 minutes postdose, when plasma concentrations were approaching their maximum values.

5.3 Preclinical Safety Data

Nonclinical data with cyclopentolate reveal no special hazard for humans based on conventional studies of single- and repeated-dose toxicity.

6 PHARMACEUTICAL PARTICULARS

6.1 List of Excipients

Benzalkonium Chloride 0.01%, Boric Acid, Disodium Edetate, Potassium Chloride, Sodium Carbonate and/or Hydrochloric Acid (to adjust pH), Purified Water.

6.2 Incompatibilities

None known.

6.3 Shelf Life

36 months from the date of manufacture.

6.4 Special Precautions for Storage

Store below 30°C. Keep out of reach of children. Discard one month after opening.

6.5 Nature and Contents of Container

1% in 15 ml plastic DROP-TAINER™ Dispensers.

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